PRE-RELEASE

The Wrong Clock

Why the Medical plan should follow the evidence, not the corporate calendar.

Christian Wasmer & Friederike Winter · Consilience · September 2026

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1 · The annual plan

The annual Medical plan is produced on the corporate calendar. It is drafted during the budget cycle, signed off before the year begins, and scheduled for revision twelve months later. Its content is fixed without reference to when the relevant science will change.

For example, a hematology-oncology team builds its Medical strategy and plan through the first half of the year, secures leadership sign-off around mid-year, and submits its budget for the upcoming year in August. In December, the field's main congress reports new data, both the team's own and its competitors'. The results are favorable, but much of the positioning the team had settled months earlier was argued without the data that would define it, and by the time the plan is put into action in January, it is already outdated. The team is then due to begin the same exercise again.

The pattern is general. Medical Affairs exists to interpret scientific evidence and convey it to the medical community, and that evidence keeps its own schedule: trial readouts, congress disclosures, guideline revisions, label changes, and observations from the field.

The plan is revised for reasons unrelated to the evidence it exists to direct, and left unchanged when that evidence moves. In many cases, after sign-off it is consulted little.

This is not resolved by the insight-gathering already in place. Continuous landscape monitoring and mid-year adjustment are established practice, but they sit on top of a structure that remains annual: the plan is rebuilt once a year, and the updates between rebuilds are treated as exceptions to it. Here, we argue that this order should be reversed. The science, not the corporate calendar, should set the schedule the plan is built on.

2 · A plan built to change

A Medical plan should have durable elements: the asset's aspirational narrative, the medical community it addresses, and the rules governing what is revised and by whom. These are established once and revised only in exceptional cases (see Section 3). Strategy and tactics need adjustment more often. The depth of each revision matches the scientific signal: most affect materials and tactics, fewer affect strategy, and the aspirational narrative is revised rarely. If these updates are considered part of a living Medical plan, the annual cycle is retained only for what belongs on a yearly schedule: budget reconciliation, retrospective review, and corporate administrative requirements. Where possible, that cycle is timed to a major scientific congress rather than to the fiscal date alone.

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This is feasible because the annual Medical plan serves three functions that convention has combined: it justifies budget, aligns the organization, and provides the scientific positioning. The corporate system places all three on an annual cycle; only the first requires it. Budget reconciliation is bound to financial reporting and the administrative calendar. Organizational alignment can be maintained against events rather than dates. Scientific positioning is continuous, revised at defined points rather than annually. Separating the three removes the reason the strategy must follow the fiscal cadence: the budget remains annual, the strategy does not.

The plan is built toward a point in the future that the evidence does not move: bringing the therapy to patients and improving their care and outcomes. That point is not itself a planning level; it is the reference the levels are set against, made specific in the aspirational narrative. Between here and there sits a sequence of scientific milestones, first approval among them, which are the waypoints on the route, not the destination. Holding the destination fixed is what creates room: the route can be revised against incoming evidence without reopening what the plan is for, which is why most signals land on tactics and few reach the aspirational narrative.

Box A · The levels

A Medical plan is structured into four hierarchical levels: the aspirational narrative, strategy, tactics, and materials. Governance is the asset-agnostic layer: the levels, signals, thresholds, and decision rights a company uses, consistent across its assets and set once.

Level	What it defines	Changes	Decided by
Governance	which levels, signals, thresholds, and rights are used (asset-agnostic)	set once	org-level
Aspirational narrative	the scientific position to establish	rarely	leadership and full Medical team
Strategy	the path, and what is required to follow it	sometimes	full Medical team
Tactics	the activities	often	the relevant subfunction
Materials	the content, data, and references used	continually	the relevant subfunction

> **A note on the aspirational narrative**
> To be workable, it must be concrete enough to yield testable hypotheses. If it cannot be decomposed into positions that data could confirm or falsify, it states an intention rather than a scientific position.

3 · From signal to execution

Integrating insights into the Medical plan follows a three-step process — assessment, decision, and implementation — that begins once a signal has been detected (Figure 1).

Signal detection is achieved through the processes already in place: for discrete events such as congresses, competitor readouts, and guideline updates, a medical intelligence function shares the updates; for continuous insights, such as those from the field or from Medical Information requests, the relevant teams analyze incoming insights at a set cadence, such as monthly or quarterly reviews.

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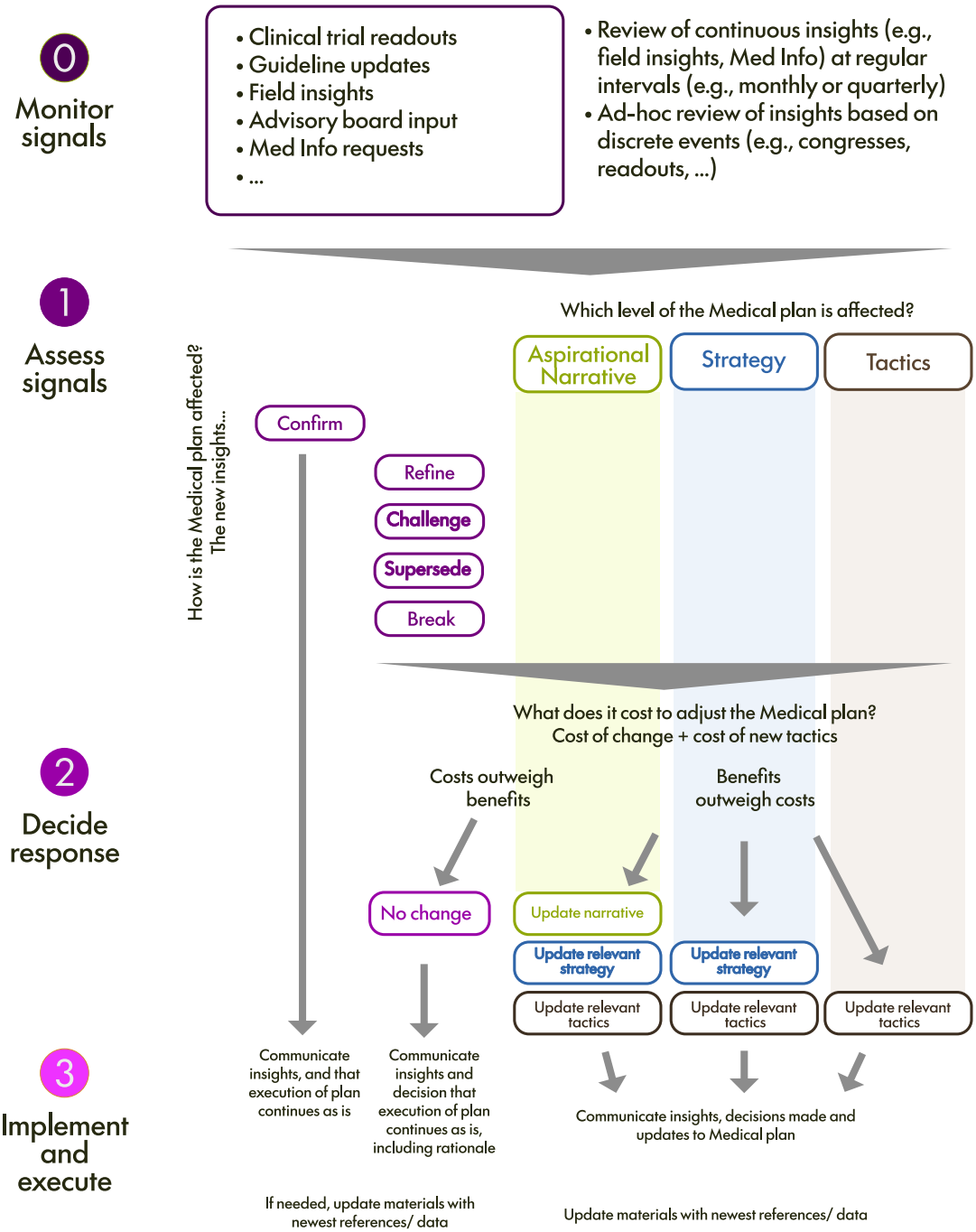


Figure 1 · From signal to execution. *The four stages by which an incoming insight reaches the Medical plan, from continuous monitoring through to execution.*

How to read this figure: *Monitoring (0) runs continuously through channels already in place — trial readouts, guideline updates, field insights, advisory board input, Medical Information requests — reviewed either at a set cadence or ad hoc around discrete events. Assessment (1) answers two questions at once: how the insight bears on the plan (confirm, refine, challenge, supersede, break) and which level it reaches (aspirational narrative, strategy, tactics). A signal that only confirms the current position exits directly to implementation. Everything else passes to the decision (2), which weighs the cost of change plus the cost of new tactics against the benefit. Where costs outweigh benefits, the plan is left as it stands; where they do not, the revision cascades downward — a change to the narrative pulls strategy and tactics with it, a change to strategy pulls tactics. Implementation (3) always includes communication, whether or not the plan itself changed.*

Note: the diagram labels the monitoring stage 0 because it precedes the three-step process described in the text rather than forming part of it.

The first step is assessment: which level the signal affects, and how. A signal confirms the current position, refines it, challenges it, supersedes it, or breaks it, and it does so at the level of materials, tactics, strategy, or, rarely, the aspirational narrative.

The second step is the decision for or against the adaptation of the plan, and it is different in kind. It weighs the cost of change against the value of the response. Cost here means organizational effort as well as money. A running trial, a committed publication plan, or an approved field strategy cannot be altered "for free," and that constraint, together with the depth of the proposed change, determines what the change costs.

The distinction separates two questions that are often discussed as one. "No change is needed" is an assessment: the evidence does not move the position. "No change is worth making" is a decision: the evidence moves the position, but the cost exceeds the value. A team that has not separated the two will debate the evidence when the disagreement is about cost. Identifying which of the two is in question clarifies much of the disagreement in stalled discussions, where the conclusiveness of the evidence is reopened in order to defer a costly decision.

Most signals confirm or refine the existing position and are resolved at the level of materials and tactics: materials are updated, field emphasis is adjusted, and the strategy stays untouched. The response scales with the assessment, from no action to updated materials, a tactical change, a strategic change, or a revision of the aspirational narrative. A change at one level requires the levels that depend on it to follow, while the levels it depends on remain intact: revising the aspirational narrative resets strategy and tactics; a tactical change resets neither.

Revisions to the aspirational narrative are rare, and not only because change at that level is expensive. A narrative chosen well is durable: it is framed to withstand the evidence likely to arrive, so most incoming evidence confirms or refines it. It can be broken outright — a pivotal trial fails and a claim the position rests on is disproved — but that is uncommon, and it leaves little to decide. The more frequent case is supersession: an assumption beneath the narrative is overtaken, a new standard of care emerging, for instance, and whether that reaches the narrative itself depends on how much of the position rested on it. Here, a narrative the evidence has superseded may still be held for a time. But that should be a decision on the record, not an assessment that never raised the question.

Finally, once decided, changes are written into the plan, the affected downstream materials are updated, and execution continues against the revised plan. Clear communication of these updates is critical to maintain the team's buy-in and keep execution aligned.

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Table 1 · Worked triggers

Example signals, each carried through the assessment and decision steps.

Signal	Arrival	Level to change	Direction	Decision (illustrative)
Confirmatory real-world result, consistent with the position	discrete	materials	confirms	update materials; no re-plan
Educational event feedback is highly positive and shows impact on patient care	discrete	tactics	refines	roll out the educational event to additional stakeholders
Competitor Phase 3 reads out positive in an adjacent indication	discrete	strategy	challenges	reprioritize emphasis across evidence generation, field, and communication; may be budget-neutral
Field observation of an emerging use pattern	continuous (reviewed on cadence)	strategy	refines	investigate; revise now or hold for the next periodic review
Guideline body signals a change in standard of care	discrete	aspirational narrative	supersedes	leadership decides whether and when to revise the narrative, and communicates the decision to the team

Illustrative. The reading depends on the asset and the situation.

4 · Scope and limits

The system in operation

The proposed approach to Medical planning has two components that behave differently. Monitoring the landscape for relevant signals is comparatively inexpensive, and it is useful wherever the science moves at all, because it can confirm stability rather than assuming it. Response — revising the plan — is the costly component. The two should be regarded separately, and the ratio between them is not fixed: a team can run continuous monitoring while keeping most of its response on the annual cycle, or set the threshold low enough that a large share of signals result in a change. That setting is a choice, and it is the main thing a team needs to calibrate.

Set the threshold too low and continuous monitoring becomes constant revision — which can indicate miscalibration rather than a volatile environment, and is corrected by raising the bar. Set it too high and small changes accumulate into a shift much larger than anticipated from the individual updates. The mitigation is a periodic review against a logged record of triggers, which makes the accumulated position visible even though no individual step was large enough to surface it.

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Another potential failure is ambiguous responsibility. If it is unclear who decides whether a signal warrants escalation, or if too many people must agree, the assessment stalls regardless of how well the threshold is set. Predefined decision rights are the mitigation, and they belong in governance rather than in the asset plan.

A further consideration belongs here: for many companies, the annual budget is fixed at the start of the fiscal cycle and does not expand because strategy has moved. In this case, what a strategic revision changes is the allocation within that fixed total: tactics are reprioritized, and the funding freed by scaling back or dropping lower-priority activities is reallocated to the ones the revised strategy now requires. This reallocation sits within budget authority that most organizations already have, without reopening the total. The total itself is revisited only when the trigger is large enough to warrant it, and that case is made through the same process the organization already uses to request budget, rather than as a standing exception.

Similarly, existing legal processes need to be integrated, though this is not always a problem: legal review applies per revision rather than annually. Each revision is smaller and more clearly motivated, which shortens individual reviews, but a larger number of revisions increases the total documented material subject to disclosure or discovery. The net effect depends on company requirements and should be checked upfront.

Adopting it

Because monitoring and response are priced separately, the question is not whether a team adopts the approach, but how far. Where signals are frequent and consequential — assets in active scientific development, peri-approval, or early in a competitive market — the response machinery earns its cost. Where they are infrequent, as for mature products in non-competitive areas, generics, and biosimilars, monitoring is still worth running, but there is little to respond to, and the annual cycle remains a reasonable place for most revisions.

Against that sits a cost the fit argument does not remove. Changing an established planning process means changing how a large number of people work, and that is expensive wherever it is attempted. It is the reason a low-signal organization can rationally keep what it has: not that the approach would fail, but that the improvement is too small to justify the disruption.

Where it does fit, the approach requires a flexibility that many organizations have lost as they have grown: the ability to adjust course, and the decision rights and within-year budget reallocation described above. These are what allow a revision to be identified, decided, and funded at the speed the evidence demands. Without them, a trigger-based plan reverts to an annual one within a cycle, because each revision that cannot be resolved quickly is deferred to the next scheduled point.

5 · The plan in use

A plan whose cadence is set by the evidence rather than by corporate requirements behaves differently from an annual one. It is stable enough to serve as a reference at any time, because it is revised when the evidence requires. Its cadence follows the science it interprets and the milestones on the way to approval and beyond. The annual cycle continues to do what it is suited to: reconcile the budget and record the year. Planning to the evidence is feasible and, we believe, superior to the annual approaches in use today.

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